Leuprorelin 22.5mg Therapy- 12 weeks

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
Treatment of hormone dependent advanced prostate cancer.	C61	00479a	N/A
Treatment of high-risk localized and locally advanced hormone dependent prostate cancer in combination with radiotherapy.	C61	00479a	N/A

* This applies to post 2012 indications

TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patients individual clinical circumstances.

Leuprorelin 22.5mg is administered subcutaneously once every 12 weeks until disease progression or unacceptable toxicity develops.

The injected solution provides continuous release of leuprorelin acetate over a three-month period.

Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	Leuprorelin	22.5mg	SC	n/a	Every 12 weeks

ELIGIBILITY:

- Indications as above

CAUTIONS:

- In patients with a history of or risk factors for QT prolongation and in patients receiving concomitant medicinal products that might prolong the QT interval physicians should assess the benefit risk ratio

EXCLUSIONS:

- Hypersensitivity to leuprorelin or any of the excipients
- As sole treatment in prostate cancer patients with spinal cord compression or evidence of spinal metastases

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant with expertise in the treatment of prostate carcinoma.

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TESTS:

Baseline tests:

- FBC, renal and liver profile
- Bone profile
- Blood glucose

Regular tests:

- FBC, renal and liver profile as clinically indicated
- Blood glucose and bone profile as clinically indicated

Disease monitoring:
Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- No recommended dose modifications

Table 1: Dose modification of leuprorelin in renal and hepatic impairment

Renal Impairment	Hepatic Impairment
No dose modification necessary	No dose modification necessary
Recommendations as per previous NCCP regimens	

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

Leuprorelin-Minimal **(Refer to local policy)**

PREMEDICATIONS: None

OTHER SUPPORTIVE CARE:

Calcium and vitamin D supplementation **(Refer to local policy).**

ADVERSE EFFECTS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.

DRUG INTERACTIONS:

- Current SmPC and drug interaction databases should be consulted for information.

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REFERENCES:

1. Loblaw DA, Virgo KS, Nam R, et al. Initial hormonal management of androgen-sensitive metastatic, recurrent, or progressive prostate cancer: update of an American Society of Clinical Oncology practice guideline. J Clin Oncol 2006;25(12):1596-1605
2. Leuprorelin (ELIGARD®) 22.5mg Summary of Product Characteristics. Accessed June 2025. Available at: https://assets.hpra.ie/products/Human/25689/Licence_PA0812-005-002_18102024115227.pdf

Version	Date	Amendment	Approved By
1	30/05/2018		Prof Maccon Keane
2	10/06/2020	Regimen review	Prof Maccon Keane
3	14/07/2025	Regimen review. Updated in line with NCCP standard wording.	Prof Maccon Keane

Comments and feedback welcome at oncologydrugs@cancercontrol.ie.

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